The Uninsured Decade

The one catastrophic risk you cannot buy a policy against — and the premium you can still pay

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A My4MLife Field Guide

Don't lose your identity and your dignity while you still have a choice.

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Who This Book Is For

This is a book about money before it is a book about health. That is deliberate.

It is for the person in the household who reads the statements. Who carries the umbrella policy, the key-man coverage, the term life that has never paid a dollar and gets renewed anyway, because that is what the responsible one does — buys down risk for people who are not thinking about risk. In my experience that seat is held at least as often by a woman as by a man. I am a man, and I write from my own chair; but this book is written to whoever holds the pen. If that is you, it is for you.

You will not find inspiration in here. No transformation stories, no promises that a supplement will save you. What you will find is an exposure your plan does not cover, priced out with sources, and one honest way to reduce the odds — with the odds stated as odds, never as a guarantee.

A word to the spouse. You may be the one who bought this. If so, you already know what happens in the ten years after a diagnosis, because you have watched it happen in another family and you have been unable to get the conversation started in your own. Do not open with your worry. Hand it over and

say one sentence: *Read the first six pages. If the numbers are wrong, throw it away.*

The numbers are not wrong. Every figure in this book carries its source and its date. Check them.

— Dr. TJ

A Note on the Numbers

I am going to ask you to make a decision about money, so I owe you a clean audit trail.

Every figure in this book carries a source and a year, in the text, where you read it — not in an endnote you will never flip to. Where a widely quoted number rests on stale data I say so in the sentence. Where two respectable models disagree, I do not average them; I take the more conservative one and say that I did. The full citation list is in the back, and the working version lives at my4mlife.com/science.

Bill and Cindy are a composite, assembled out of the arc that shows up over and over in the national data. Every dollar figure attached to them comes from a published survey of what that care costs. The tennis league is invented. The invoice is not.

One number I will not give you is your number. The ten-year ledger in Chapter 3 is a national-median household. If you want your own figure, my4mlife.com has a calculator that

runs this arithmetic on your household instead of a composite one.

That is the standard. Hold me to it.

Chapter 1 — The Risk You Didn't Underwrite

Somewhere in your house there is a folder, or a portal login, that holds every policy you own.

You did that. Not your accountant, not the carrier — you, or the two of you at a kitchen table. Over thirty years you built a lattice of coverage under your family so that nothing that could happen to you would land on them as a bill. And not alone: the person who put this book in your hands is the one who protects that folder. Keep them close through every page of this one.

Which policy in it pays for memory care?

Not the hospital stay — you have that. Not the rehab after the hip; Medicare covers a stretch of that, and we will get to how long. I mean the other thing: what starts as a word you cannot find in a meeting and ends, eight or ten years later, with a bed in a locked wing and someone else spooning food into your

mouth at eight thousand dollars a month, every month, until it stops.

Medicare pays nothing toward it. Not a reduced amount. Not a copay structure with a bad deductible. Zero. It is the single largest uncovered financial exposure in American retirement, and your advisor has fewer tools to hand you here than on any other line because the market itself thinned out underneath them.

I am not going to tell you that you can prevent dementia. Nobody can, and anyone who says otherwise is either careless or selling something. What I will tell you is that this is a risk, that risk is a thing you already know how to price, and that there is one more place to post a reserve against it.

Not a carrier. Not a rider. You.

First, the inventory

Risk	Policy you hold	Who writes the check
House burns	Homeowner's + rider	Carrier
Someone is hurt on your property	Homeowner's + umbrella	Carrier
Car wreck, at fault	Auto + umbrella	Carrier
You die early	Term life	Carrier
You can't work at 55	Long-term disability	Carrier
Your partner dies, business at risk	Key-man / buy-sell	Carrier
Professional mistake	E&O	Carrier
Hospitalization at 72	Medicare + supplement	Carrier
Rehab after the hip, first 100 days	Medicare Part A, conditions apply	Carrier, then you
Ongoing custodial and memory care	**none available**	**You. Then your spouse. Then Medicaid.**

Every line but one resolves to somebody else's balance sheet. That was the point of thirty years of premiums. The last line does not fully resolve. Where a policy or hybrid rider is available and you qualify, it funds part of the risk — ask your advi-

sor whether one belongs in your plan. What no contract does is lower the risk. This book is about that job.

Notice the shape of that table. The risks you covered are the low-probability ones — your house probably will not burn. You bought those policies anyway, because the loss, if it landed, would be unrecoverable, and unrecoverable is the test, not likely.

Now look at the uncovered line. A 2025 study in *Nature Medicine* — the largest lifetime-risk analysis of its kind, following more than 15,000 people from age 55 until they either developed dementia or died — puts the lifetime risk of dementia after 55 at **42%**: 35% for men, 48% for women [Fang et al., Nat. Med. 2025]. That is a coin flip that leans against you. And in a marriage you are not underwriting one life. You are underwriting the question *does this happen to either of us* — and run that arithmetic and the odds come out to roughly two in three.

You insured the fire. You did not insure the likelier thing. Not out of carelessness: in every other row of that table, a market showed up with an illustration and a pen and asked you to sign. On the last row, the market thinned out to almost nothing before most of us got there.

Self-pay, then spouse-pay, then impoverishment. That is the actual coverage structure on the largest exposure in your retirement.

Who holds the pen

One more thing about that folder, because I would be lying to you by omission if I left it out. I am a man, and I write from that chair. But when I look at the households who actually do this work — and when I listen to the estate attorneys who sit across the table from them — the person holding the folder is very often not the husband. It is the wife, or the partner. She keeps the documents. She knows the advisor by first name. She is the one who signs the forms. And in a great many households she ends up making every one of these decisions alone, because her husband is the one whose mind is going, and he can no longer sit at that table with her.

She is not only the one deciding. She carries the higher lifetime risk herself — 48% against his 35%, from the same study.

So if that is you, this book was not written to your husband with you copied on it. It is written to whoever holds the pen.

Bill

Bill is seventy-one, a composite of a hundred versions of the same man. He sold his share of a mechanical contracting business at sixty-eight, plays doubles on Tuesdays and Thursdays, and has been married to Cindy for forty-four years. Cindy kept the books for that business for twenty-six of them, and the household's for all forty-four. When the advisor calls, the call is for her.

Two years ago he tore his rotator cuff reaching for a low forehand. Repaired, five months of physical therapy, back on the court by spring, a few thousand out of pocket; Medicare and the supplement absorbed the rest, exactly as designed. That is what a covered risk looks like from the inside: expensive, disruptive, and then it ends, and the balance sheet closes back up over the wound.

A rotator cuff changes your tennis game. Cognitive decline changes who pays the bills, who drives, who remembers which grandchild is which — and hands Cindy a job she never applied for: nurse, guardian, sole signature on every decision the two of them used to make together, and eventually the only one in the marriage who remembers the marriage.

The shoulder was insured. The memory is not.

This is an audit finding, not a scare

What I have handed you is not a prognosis. It is an audit finding — one line item, on a coverage schedule you built yourself, with the thinnest entry in the second column. You already know how to respond to one. You size it, you decide whether it is material, and if it is, you fund it.

One promise before we go on. I am not going to tell you at the end that this pays for itself. When I run the expected-value math in front of you later, it does not break even against de-

mentia costs alone. With the odds at roughly two in three for a couple it gets close — closer than it has any right to be — but close is not break-even. This is a ruin-avoidance trade, not an investment, and I would rather lose you in Chapter 7 than get you there on a number I had to bend.

Chapter 2 — What Nobody Pays For

The most expensive assumption in American retirement planning is that Medicare handles this. It does not. Not partially.

Here is the operative language, from Medicare's own page on long-term care:

> *"Medicare doesn't cover custodial care if it's the only care you need."*

[Medicare.gov, current]

Everything turns on one word. **Skilled care** requires a licensed professional — wound management, IV therapy, rehabilitation with a documented trajectory of improvement. Medicare covers that. **Custodial care** is help with the activities of daily living: bathing, dressing, toileting, transferring, eating, and supervision to keep a person from wandering out the front door at two in the morning. It does not require a

nurse. It requires a person in the room, for as many hours as the disease demands.

That second definition is a nearly exact description of what dementia care *is*, from the middle stage onward. The category of care this disease needs is the category the program specifically excludes. That is not an oversight: Medicare was designed in 1965 as acute medical insurance. It was never built to do this.

Medicare does cover hospitalization, physician visits, imaging and cognitive testing, Part D drugs, hospice at the very end, and up to 100 days of skilled nursing per benefit period. It does not cover memory care, assisted living room and board, in-home aides, adult day programs, or any long-term stay that is "only" custodial.

That 100-day line does the most damage, because people remember the number and forget the conditions. It is *skilled* nursing facility care, gated by a qualifying hospital stay, conditional on a clinician continuing to document that the care is skilled, and carrying substantial daily coinsurance from day 21. In a dementia course it is usually irrelevant: it gets consumed by the hip fracture or the pneumonia, and then the person goes to memory care. No benefit period restarts for "still has dementia."

What the private-pay meter reads

If Medicare is not paying, someone is. National medians, with the year on each figure:

- **Memory care:** $8,019 per month, 2026 — about $96,000 a year. In Texas, $6,684 [SeniorLiving.org 2026].
- **Assisted living:** $6,200 per month, or $74,400 a year, up 5% year over year [CareScout 2025].
- **In-home caregiver:** $35 an hour. At 20 hours a week that is $36,400 a year; at 44 hours a week, $80,080 [CareScout 2025].
- **Nursing home, private room:** $129,575 a year — $355 a day [CareScout 2025].

Spend-down

One government program does pay for long-term custodial care: Medicaid. It is means-tested, which means the way you become eligible is by not having money anymore. The industry word is **spend-down**. Numbers vary by state; 2026 Texas figures are illustrative, but the architecture is the same nearly everywhere [TX Medicaid, 2026 figures]:

- **Countable resources** must come down to roughly $2,000 for an individual applicant, or $3,000 for a couple.
- A **five-year, sixty-month look-back** on asset transfers. Gifts inside that window trigger a penalty period of ineligi-

bility — the provision that punishes families who start planning after the diagnosis instead of before it.

- The at-home spouse is not left with nothing. A **Community Spouse Resource Allowance** protects between $32,532 and $162,660 in assets, plus up to $4,066.50 a month in protected income.

All of that is legal and financial work, not health work, and none of it is do-it-yourself. Medicaid planning, spousal protections, titling, the durable power of attorney and the healthcare directives signed while they can still be signed — that is your estate attorney's and your advisor's craft. The five-year window makes it timing-sensitive: it belongs on their calendar now, not after a diagnosis closes the good options.

The market that left

Around the year 2000, more than a hundred carriers sold standalone long-term-care insurance in the United States. Today fewer than a dozen actively write new standalone policies [Am. Academy of Actuaries]. That is not a market correction. That is an evacuation.

Most of these contracts are guaranteed renewable but not premium-guaranteed: the carrier cannot cancel you, but it can ask your state insurance department to raise the rate on the entire class — and it has, repeatedly. Genworth received a 68.5% average approved increase in a 2026 Nebraska filing; in 2022 it

raised rates on more than two thousand policyholders by an average of 97%, with individual increases from 79% to 173%; a 161% request was denied in Massachusetts as "unjust, unfair and inequitable" [S&P Global; state filings, 2022–2026]. John Hancock secured increases across more than ten states in a single 2026 quarter, on blocks already carrying 15% in 2012–15 and 43.8% phased from 2020 [S&P Global; longtermcare-desk.com, 2026]. Stack those and a policyholder at seventy may be paying two or three times the premium signed for at fifty-eight, at exactly the point when the household's income is fixed.

That is the trap in one line. **The event that makes you want the policy is the event that makes you uninsurable for it.**

So the honest count. Only about 10 to 15% of Americans 65 and older hold long-term-care insurance of any kind, against an estimate that roughly 70% of people over 65 will need long-term care of some sort [CRR / HCG Secure, 2026]. Seventy percent will need it. Ten to fifteen percent have it. You are not lazy for being in the uncovered majority; the market moved out from under a generation of buyers.

None of that is an indictment of the people who advise you. It is the terrain they now plan across. When a hundred carriers become a dozen, the professional holding your plan has fewer instruments and harder tradeoffs than the one who sat across from your father — which is exactly why their review matters

more now, not less, and why this book's question belongs on that agenda.

I do not sell insurance and I cannot advise you on it. If you are shopping it, your advisor or a licensed agent should answer three questions in writing: is the premium guaranteed or merely guaranteed renewable, and what is this carrier's rate-increase history; what triggers a benefit, who decides, and how long is the elimination period you pay through; and what does the benefit look like in the twelfth year against the projected cost of care, not today's? A policy or a hybrid rider can fund this risk; it cannot lower it, and the work in the rest of this book cannot fund it — some households should own both, so ask your advisor.

Chapter 3 — Ten Years, One Household

Bill is diagnosed at seventy.

It happens the way it usually happens: eighteen months of Cindy noticing, six months of Cindy not saying anything, one bad afternoon when he cannot find his way back from the club he has driven to for eleven years, and then a neurologist's office and a word.

People with Alzheimer's commonly live eight to ten years after diagnosis. So the question is not *what does a diagnosis cost* — a diagnosis costs a copay. The question is what the decade costs.

Stage	What the care actually looks like	Estimated family cost
Years 1–3 (age 70–73) Early	Cindy provides nearly all care. Doctor visits, medications, home safety changes, legal and financial planning. Average dementia-caregiver out-of-pocket spending runs about $12,388 a year — *a figure based on 2021 data, and meaningfully higher in today's dollars* [Alz. Impact Movement, 2021 data].	**~$35,000–$40,000**
Years 4–6 (age 73–76) Middle	Cindy can no longer do it alone. Paid in-home caregiver 20 hours a week at $35/hr (~$36,400/yr) [CareScout 2025], plus an adult day program two days a week (~$9,900/yr), plus out-of-pocket medical.	**~$170,000–$180,000**
Years 7–10 (age 76–80) Late	24/7 supervision required. Memory care runs $8,019/mo nationally, about $96,000 a year [SeniorLiving.org 2026]; a private nursing-home room is $129,575 a year [CareScout 2025]. Assume roughly 3.5 years in a facility.	**~$290,000–$340,000**
Ten-year total	Cash and care costs falling on the household — before counting the healthy spouse's unpaid hours, lost income, or health decline.	**~$495,000–$560,000**

A half-million-dollar number invites the suspicion that somebody built it backward from the number they wanted, so here is the check. The Alzheimer's Association puts the average total lifetime cost of care for one person with dementia at **$405,262**, in 2024 dollars, with **70% of it borne by the**

family through out-of-pocket spending and unpaid caregiving [Alz. Assoc. 2026]. Our composite lands above that national average, which is what you would expect: a ten-year course with 3.5 years in a facility, against a figure that averages every course, including the short ones.

Why the ledger is shaped like that

In years one through three, the household is buying *things*. Medication copays. A grab bar. An attorney's time — Cindy makes the call — to get the powers of attorney signed while Bill still can. A bad year financially, not a catastrophic one — which is exactly why nobody restructures anything, and why the first three years give a false read on the whole decade.

In years four through six, the household starts buying *hours*. This is the phase change. Somewhere in there is a specific week where Cindy admits she cannot leave the house to go to the grocery store, and the household starts paying $35 an hour for someone else to be in the room. Twenty hours a week is one shift a weekday and costs $36,400 a year; every additional hour is another $1,820 a year, forever. Care is the one purchase in American life with essentially no economies of scale.

In years seven through ten, the household is buying *a bed with staff attached*, and the meter reads $96,000 to $130,000 a year until the day it stops. That is when the portfolio conversation happens, and by then it is a liquidation sequence, not a planning conversation.

Which account it comes out of

It does not come out of income. Social Security and whatever pension the household has covers essentially none of a $96,000-a-year care bill on top of ordinary retirement living. The shortfall comes out of the portfolio, on the facility's schedule rather than the market's, which makes Bill and Cindy forced sellers every month for years — the sequence-of-returns risk their advisor explained at sixty-two, arriving at seventy-four at a withdrawal rate nobody modeled. Then the tax layer: realized gains, IRA withdrawals taxed as ordinary income, higher Medicare surcharges. Then the house, sold in year seven or eight at the moment Cindy has the least capacity to run a sale. None of that shows up in the right-hand column, and all of it turns a $500,000 care cost into something considerably worse.

The tail on the right

Bill and Cindy are a median household with a median course of disease, and this distribution is not symmetric. The downside is bounded — a short course, two or three years, mostly at home. The upside is not. Younger, physically robust patients live longer with the diagnosis, which is the cruelest arithmetic in this subject: the fitter the body, the longer the decade. A course that runs twelve or fourteen years, five of them in a private room at $129,575 a year in a high-cost metro, does not

produce a $560,000 bill. It produces something well north of $800,000, with no ceiling written anywhere.

The one that isn't on the statement

In 2025, roughly 13 million Americans provided more than **19 billion hours** of unpaid dementia care, valued at over **$446 billion** [Alz. Assoc. 2026]. Divide that down to one household and it is Cindy, working an unpaid second full-time job for a decade — on call at three in the morning, with no version where she gives notice. If you priced Cindy's hours at the same $35 an hour the agency charges, the ten-year number above roughly doubles.

This is where most books about this subject would move on to solutions. I am not going to, because the financial case is not the case. The largest cost in this decade is not on the ledger and it is not paid by Bill.

Chapter 4 — The Second Patient

Cindy is seventy-nine when it ends.

She has been the primary caregiver for ten years, the last three driving to a facility every day because she could not stand the thought of him being there without a familiar face in the afternoon. She is the one who learned to give the medications. She is the one who took the car keys, which was the worst day of the marriage.

And she is the second patient in this story. She was never diagnosed with anything, never entered into a chart. Nobody ever wrote her a treatment plan. If you read only one chapter of this book, read this one.

A landmark 1999 JAMA study — Schulz and Beach, the Caregiver Health Effects Study — found that elderly spousal caregivers reporting strain had a **63% higher four-year mortality rate** than non-caregiving spouses of the same age [Schulz & Beach, JAMA 1999]. That study is twenty-seven years old, and I am telling you its age on purpose. Nothing since has overturned it, but hold it as what it is: a robust, old,

still-standing finding that the strain of caregiving is associated with dying sooner.

The diagnosis does not take one life. It taxes two.

The rest of the picture is consistent. Family caregivers report chronic conditions at nearly twice the rate of non-caregivers, **45% versus 24%**, and roughly **40% of dementia caregivers** show clinically significant depressive symptoms, against 5 to 17% [Family Caregiver Alliance; treat as older figures]. And they stop taking care of themselves, which is the mechanism connecting all of it: in the Parkinson's data, **34% of care partners canceled or missed their own healthcare visits** and more than one in five retired early or cut work hours [Parkinson's Foundation, 2024 data].

The decade, from her side of it

Year two, Cindy is still calling it *helping*. She drives to the club now because Bill got turned around once, and she has framed it as wanting the time together. The decisions the two of them used to make at the kitchen table, she now makes alone, and signs for both of them. Year four, the calendar on the refrigerator has three colors of ink on it and none of them are hers. She goes to the wedding and leaves before the reception, because three hours away is a project and six is not possible — and the invitations thin out, not because anyone decided anything, but because she has said no eleven times.

Year six, she is not sleeping. He is up at two, and then at four, and she is listening for him even in the hours he is down. She has a cardiology follow-up she moves twice and then lets go. The aide comes four hours a weekday, enough to get to the grocery store and back and not enough to get to a gym, and she has not lifted anything heavier than a laundry basket in four years. Her daughter says *Mom, you have to take care of yourself*, and Cindy says *I know*, which is true, and does nothing about it, which is also true.

Year eight, he is in the facility and she is there every afternoon, and she has come to understand something she cannot say to her children: that the hardest part of this was never the money. It was the year she was grieving a man who was still sitting at the table.

> *She did not get the diagnosis, and she is paying for it longer than he did.*

Who this book is actually for

If you are the person who reads the statements, you have spent thirty years buying down risk on behalf of the person across the table. That is what the umbrella policy was. That is what the term life was — you were never going to see a dollar of it. You bought it so that the worst version of your absence did not land on the one you love as a bill.

The uninsured decade is the one exposure where that instinct runs short of instruments. Coverage, where you can get it, funds part of the bill; no contract lowers the odds of the decade itself. And if it goes unaddressed, the outcome is the precise thing you spent three decades insuring against: the worst version of what happens to one of you, landing on the other — not as a check to cash, but as a decade to spend.

And if you are the Cindy in your house — the one who already keeps the folder, and can already see which chair you may end up in — read this chapter twice. The decade lands on whichever of you is left holding the pen, and at two in three for a couple, you do not get to choose which chair. That is the whole argument of this book. Everything after this is arithmetic and logistics.

Chapter 5 — The Dividend

Every policy in that folder has one thing in common, and it is the thing nobody says out loud at the closing table.

You hope it never pays.

The premium in this book has a different shape. It has two payouts.

The first is the one you have been reading about for four chapters. It is contingent. It arrives, if it arrives at all, as a decade that did not happen — a ledger that never got written, an aide who was never hired, a Tuesday afternoon that Cindy spent at her book club instead of in a parking garage. You will never be able to prove you collected it. You will only be able to notice, at eighty-two, that the invoice never came.

The second payout is not contingent, and it does not wait.

It starts inside about six weeks. It is paid in sleep that goes unbroken, in the fact that you can pick up a grandchild without doing the calculation first, in an afternoon at four o'clock that does not require a decision about coffee. It is paid in a waist-

band, a lab report, a sex life that did not quietly end, a walk up a jetway with a bag over your shoulder.

You do not have to develop dementia to collect it. You do not have to *avoid* dementia to collect it. It pays on the premium itself.

It is the only policy in the folder that pays before the claim.

What you are actually buying

And it is the part I actually care about, so let me say it once and get back to the arithmetic. The goal is not to avoid dementia. It is not even to protect the money, though the money is why we are talking. The goal is the best mind you can have until your last day of life — the whole span, not the tail. Sharp at seventy-eight. Present at eighty-four. Still the one people call when something has to be decided. The long *useful* life, with the decline compressed into as short a window as the body will allow, at the far end, where it belongs.

The dividend, itemized

Sleep. Not eight hours as a virtue — seven or eight that are *consolidated*, so you wake at six and the day starts, rather than waking at six having been up at one-forty and again at four.

Strength. The kind that shows up as capacity: two suitcases up a flight of stairs, getting off the floor without a hand on the coffee table. Muscle is the reserve you draw on in every bad week of the next thirty years.

Energy at four in the afternoon. The one your spouse notices before you do. The four-o'clock collapse is not aging. It is usually blood sugar, sleep debt, and a lunch that was mostly bread.

Sex. Not a lifestyle amenity — the vascular system filing an early report, and the most reliable canary either partner in a marriage gets. It is also, plainly, a payout.

Presence. Your grandchildren will not remember whether you were fit. They will remember whether you were *there* — and "there" is a metabolic state before it is a character trait.

The active retirement decade. Sixty-eight to seventy-eight is the decade you were saving for, and most people spend it at a level of function they never chose and never noticed choosing.

Not one line on that list requires a diagnosis or a statistic. Every line pays out inside a year.

The Premium

The premium: *roughly ten hours a month and roughly eight to ten thousand dollars a year, most of it money you are already spending. Chapter 6 prices it out line by line.*

What it buys: *two payouts from one premium. A contingent one — lower odds on the decade in Chapter 3, stated as odds and never as a promise. And a non-contingent one, paid out while you are alive to spend it: sleep, strength, energy, sex, presence, an active retirement decade.*

What it does not buy: *immunity. Nothing in this book prevents, cures, or reverses dementia. Anyone who tells you otherwise is selling something, and Chapter 7 is where I show you the math with nothing hidden.*

What a dividend would have looked like for Bill

Bill was fifty-five once. Thirty pounds heavier than at forty. An A1c that came back at 5.9 that spring, which the nurse called *borderline* and he heard as *fine*. Sleeping badly for years and

calling it a personality trait. Off the tennis court because of a knee he never got looked at.

Nobody can tell you what that does to the diagnosis at seventy, and Chapter 7 is where I stop hand-waving. But this I can say with no statistical hedging at all: the thirteen years between fifty-five and sixty-eight would have been materially better years, and those years are contingent on nothing. That is the dividend, paid whether or not the claim is ever filed.

And it is why the line on the cover of my first book belongs in this one too. Don't lose your identity and your dignity while you still have a choice. The ledger is about the money. The dividend is about the choice.

Chapter 6 — Ten Hours and $675

Ask someone in their fifties why they are not doing this and the answer is never the money. It is time — a genuine constraint, not an excuse. So let's find the actual number, because it is not the one most people carry around. That one was set by a magazine article in 1998.

Two strength sessions a week, forty-five minutes each. Not five. Two. Compound movements — a push, a pull, a hinge, a squat, a carry — loaded heavily enough that the last two repetitions are genuinely difficult, which is the entire active ingredient and the part almost everybody leaves out. Two sessions done seriously beat five done conversationally.

The fasted morning walk: twenty minutes, five mornings, before you eat. A walk taken fasted does something metabolically different from the same walk after breakfast, and it is the single most accessible lever most people have. Outside if you can, for the light. The protein rule attaches here: **you break the fast after the walk, and you break it with thirty to forty grams of lean protein before anything**

else goes in. Not the coffee-and-toast that has been the default since 1974.

The gross number and the net number

Done honestly, the gross number is not ten. The strength sessions are six and a half hours a month, the walks another seven, the planning one — roughly fifteen before a single minute of cooking. The title works because of what those hours displace, so here is that subtraction in the open.

Net new time, honestly counted: **about ten hours a month. Two and a half hours a week** — roughly two percent of your waking hours, less than the average American your age spends watching television in three days. If your calendar cannot absorb ten hours a month, the calendar is the finding, not the protocol.

The basket

Line item	Low	Mid	High
Gym membership, mid-tier	$30/mo	$60/mo	$120/mo
Personal trainer, once a week	$160/mo	$280/mo	$560/mo
Grocery premium — better protein and produce, two-person household	$120/mo	$185/mo	$250/mo
Supplement stack	$100/mo	$150/mo	$200/mo
Lifestyle subtotal	**$410/mo**	**$675/mo**	**$1,130/mo**
Annualized	**~$4,900/yr**	**~$8,100/yr**	**~$13,600/yr**
Optional: one compounded prescription therapy where indicated and prescribed	$150/mo	$225/mo	$350/mo
Annualized, with one prescription therapy	**~$6,700/yr**	**~$10,800/yr**	**~$17,800/yr**

[premium basket pricing, 2025–2026]

Three notes. **The trainer line is optional and the largest single item** — drop it and the mid column falls to about $4,700 a year. I keep it because heavy compound work done with bad mechanics is how people in their sixties get hurt and quit. **The grocery line is a delta, not a total.** And **the**

prescription line is one item, not five — a household typically uses none of these categories or one, and which, if any, applies to you is determined by an independent licensed prescriber after labs and an actual clinical evaluation.

So: **roughly $8,000 to $10,000 a year for the lifestyle-only version**, and **roughly $15,000 to $18,000 a year if a prescription therapy is part of it.**

Now subtract what you are already spending. Most households in this income band already carry a gym membership, frequently unused. You are buying food regardless, so the grocery line is only the quality delta, and it drops as restaurant meals drop. Most households already have a shelf of supplements bought without a plan and mostly not taken. Run the mid column with that subtraction and the genuinely new money is three to five thousand a year, not eight. The rest is reallocation — the same dollars, aimed. The money is not missing. It is unmanaged.

The premium against the face value

- **Premium:** roughly $10,000 a year. **Ten-year premium:** $100,000.
- **Exposure:** the ten-year ledger in Chapter 3 — roughly $495,000 to $560,000 for a median household, with a right tail past $800,000 and no ceiling written anywhere.

Roughly five to one at the median exposure, and better in the tail — exactly the region insurance exists to address. But I will not let that pass as the same ratio as your term policy, which pays full face value if the event occurs. **This premium does not.** It pays nothing on a diagnosis. What it does is reduce the probability that the event occurs at all — by some amount, which is real, and which is smaller than the number you would like it to be. That distinction is the entire subject of the next chapter.

Which is why this basket is not being scored against anything in your folder. A funding instrument and a risk-reduction habit do different jobs on the same line, and neither substitutes for the other. Some households should own both — ask your advisor.

The Premium

The premium: *ten net-new hours a month — two forty-five-minute strength sessions a week, a twenty-minute fasted walk five mornings, four dinners cooked instead of ordered, fifteen minutes of planning on a Sunday. In dollars: roughly $8,000–$10,000 a year, or $15,000–$18,000 if a prescription therapy is indicated and prescribed. Genuinely new money, for most households: $3,000–$5,000 a year.*

What it buys: *a reduction in probability — not a benefit payment — on a $495,000–$560,000 median exposure with an unbounded right tail. Plus the dividend, which pays annually and unconditionally.*

What it does not buy: *a face value. There is no check on diagnosis, and not one of those hours can be banked, borrowed against, or paid in a lump sum at sixty-nine. It is a premium. It is due monthly.*

Chapter 7 — Odds, Stated Honestly

I promised in Chapter 1 that I would not tell you this pays for itself. Here is where I keep that promise, with the arithmetic on the page. Four inputs; argue with any of them and the answer moves.

One. Lifetime risk of dementia after fifty-five: 42%. From the ARIC cohort study published in *Nature Medicine* in January 2025, following more than 15,000 people from age fifty-five — 42% overall, 35% for men, 48% for women [Fang et al., Nat. Med. 2025].

Two. For a mixed-sex couple, the probability that at least one of you develops it: about 66%. Not a separate finding — arithmetic on the first: 1 – (0.65 × 0.52) = 0.662.

Three. Average lifetime cost of care if it happens: $405,262, in 2024 dollars, roughly 70% of it borne by the family [Alz. Assoc. 2026].

Four. Risk reduction from sustained structured lifestyle change: 30%. The number I have to defend hardest,

and I defend it below. It is deliberately lower than the figure you have probably seen quoted.

One caveat travels with the 42% every time it is used. **That 42% is all-cause dementia, including mild late-life cases** — a broader category than "Alzheimer's requiring years of full-time memory care." The $405,262 figure is the average cost among people who develop dementia and require care, not further discounted for the mild cases. So the table below is not "66% of couples face a half-million-dollar decade." It is a risk-weighted expected value, using the broadest risk figure and an unadjusted average cost, which pull in two directions at once.

	Lifetime risk	Expected cost, un-mitigated (risk × $405,262)	Expected savings at 30% risk reduction	Net against a $100,000 ten-year premium
Single person	42%	$170,210	**$51,063**	**−$48,937**
Couple — "does this happen to either of us"	66.2%	$268,283	**$80,485**	**−$19,515**

There it is. A couple paying $10,000 a year for ten years comes out roughly **nineteen thousand five hundred dollars short** on dementia-cost avoidance alone. A single person

comes out about **forty-nine thousand short**. It does not break even. I told you in Chapter 1 that it would not.

The number you have seen quoted

You have probably encountered a friendlier figure: that up to 45% of dementia cases are attributable to modifiable risk factors. It comes from the 2024 Lancet Commission, which identified fourteen — hearing loss, hypertension, smoking, obesity, depression, physical inactivity, diabetes, excess alcohol, head injury, air pollution, social isolation, high LDL, untreated vision loss, and low education [Lancet Commission 2024]. 45% is a **population-attributable fraction**: what share of cases would theoretically not occur if everyone eliminated all fourteen factors across the whole life course, starting in childhood. It is a public-health ceiling under conditions that will never obtain. **It is not a statement that you, personally, starting at fifty-eight, cut your own risk by 45%.** No study shows that.

For the record, the ceiling would put the couple's arithmetic at $268,283 × 0.45 = **$120,728**, which is $20,728 *above* the $100,000 premium. I show it so you know I did not hide it, and I label it plainly: **a population ceiling, not a result any individual household should expect.** It is the only version of this arithmetic that produces a positive number, and I am not going to sell you the positive one. **30% stays the working assumption.**

The evidence underneath 30%

The strongest single piece is US POINTER, published in *JAMA* in July 2025 — 2,111 participants, mean age 68.2, two years, randomized. Both a structured multidomain lifestyle program and a self-guided version improved global cognition, and the structured arm improved significantly more; the benefit held across age, sex, ethnicity, cardiovascular status, and APOE-ε4 genotype [US POINTER, JAMA 2025]. Read what that says and what it does not. It is a protection-from-decline finding over two years, **not** a finding that anybody prevented dementia. No trial has run long enough to show that.

Pooled across meta-analyses, physical activity carries a relative risk for all-cause dementia of about **0.80** in the more active, with leisure-time activity closer to **0.76** [Iso-Markku et al., BJSM 2022; Feter et al., Lancet Public Health 2026] — roughly 20 to 24% from one domain, observational, with all the confounding that implies. Stack a second and third and 30% is a defensible aggregate assembled across evidence types, not a number any single trial has produced. That is the honest state of the field.

Three things that tip the ledger

One: you are not buying one risk reduction. You are buying several, simultaneously. The table prices one line item. The strength work, the metabolic control, the sleep, and

the removal of chronic inflammatory load act on the same substrate that produces type 2 diabetes, hypertension, cardiovascular disease, stroke, and sarcopenia — and those conditions are not in a separate column from dementia. They are the road that runs to it. A type 2 diabetes diagnosis at sixty to sixty-nine carries a **hazard ratio of 1.70** for subsequent dementia; at fifty to fifty-nine, **1.72**; before fifty, **1.90** — each against diagnosis at seventy or later. Obesity plus diabetes diagnosed before fifty carries a hazard ratio of **3.05** [Qi et al., PLOS ONE 2024]. Roughly a tripling, from two conditions both addressed by the same ten hours a month. The math above ignores that correlated portfolio, which means **it is a floor**.

Which brings me to the sentence this chapter has been walking toward. **You do not insure the house because you expect it to burn.** The expected value of your homeowner's policy is deeply negative and has been every year for thirty years, and you have never once considered dropping it, because you are not buying expected value. You are buying the elimination of an outcome you cannot come back from. That is ruin avoidance, a completely different trade from an investment — and the uninsured decade is the last unrecoverable loss on your schedule, the one where part of the reserve has to come out of physiology because no carrier can post that part for you.

Read that as an argument for coverage, not against it. Nothing here is a reason to drop or defer anything in your folder: funding the risk and lowering it are two halves of one answer. Some households should own both — ask your advisor.

Where that leaves the trade

The Premium

The premium: *ten hours a month. Roughly $8,000–$10,000 a year, most of it money already leaving your account.*

What it buys: *a conservative 30% reduction in the odds of the decade in Chapter 3 — which does not break even on that line item alone — plus a reduction across a correlated set of exposures the arithmetic never priced, protection against a tail with no ceiling, and a dividend paid annually and unconditionally starting in about six weeks.*

What it does not buy: *immunity, a benefit check, or a guarantee. Nothing in this book prevents, cures, treats, or reverses dementia, Alzheimer's, or Parkinson's. It lowers the odds. That is the only sentence an honest person can sell you, and it is the only one I will.*

Chapter 8 — Mind: The Destination Has a Line Item

Every policy in that folder has a schedule of benefits. This one has a schedule too. It is four lines long.

Line item	What it covers	Hours a month	Dollars a month
Mind (Ch. 8)	Sleep, morning light, the gut barrier, and three front doors — hearing, vision, teeth	~0 net new	$100–$200 supplements; some one-time hardware; $150–$300 if prescription gut-barrier support is indicated
Muscle (Ch. 9)	Two strength sessions a week, the fasted morning walk, protein first, visceral fat	~9	$190–$680 gym and coaching; $150–$300 if weight-management medication is indicated
Mitigate (Ch. 10)	A1c, LDL, blood pressure, alcohol, smoking; hormones and the two canaries; air and water	~0 — mostly subtraction	labs and consults; $150–$350 if hormone support is indicated
Motivate (Ch. 11)	One named face, one sentence about who you are, fifteen minutes on a Sunday	~1	$0

What happens at night

Sleep is not rest. In 2013 a research team at the University of Rochester described a waste-clearance network in the brain —

cerebrospinal fluid moving through the spaces between brain cells and flushing out the metabolic debris of the day, including the protein fragments most directly implicated in Alzheimer's pathology [Xie et al., *Science* 2013]. That system is largely dormant while you are awake. It runs during deep, slow-wave sleep, in the first third of the night. The trash goes out at night or it does not go out. The second job of the night is filing: during REM sleep, the day you just lived gets moved from short-term storage into long-term storage. Miss it and the meeting from Tuesday is smudged at the edges by Thursday — which is, incidentally, the exact symptom that made you pick up this book.

Now the displacement I promised you in Chapter 6. The evening hour that reorganizes itself when the strength sessions arrive is usually the hour in which a second drink happens, and alcohol is sedating, which is not the same as sleep. It gets you down faster, then the suppression concentrates in the second half of the night, cutting into the REM block while you are unconscious and unable to notice [Ebrahim et al., Alcohol Clin. Exp. Res. 2013]. That is not a moral point. It is an accounting point: you are paying for the drink twice, and the second payment is drawn from the account this whole book is about.

What your gut is manufacturing

About 90% of the body's serotonin — mood, sleep, appetite — is produced in the gut. Roughly half of your dopamine — moti-

vation, reward, focus — is made there too. And the vagus nerve, the largest cranial nerve you own, is overwhelmingly a one-way street: roughly 80% of its traffic runs gut-to-brain, not brain-to-gut [Gershon & Tack, Gastroenterology 2007; Eisenhofer et al., JCEM 1997; Berthoud & Neuhuber, Auton. Neurosci. 2000]. Your gut is filing a continuous report to your head. The only question is what is in the report.

The failure mode, in one line. The lining of the gut is a single layer of cells — one cell thick — and it is the entire physical boundary between everything you eat and the rest of your bloodstream. Degrade it, and bacterial fragments cross into circulation that were never supposed to be there. The immune system reads that as an invasion and mounts a low-grade inflammatory response, everywhere, all the time. The same inflammatory traffic degrades the barrier protecting the brain. And the brain translates it into the only vocabulary it has: fog, flat mood on a perfectly fine Tuesday, the colleague's name that is gone for three full seconds. You are not necessarily getting old. You may be inflamed.

Three front doors standing open

Three of the Lancet Commission's fourteen modifiable risk factors are not what anybody expects, and all three are things a competent household fixes in a month [Lancet Commission 2024].

Hearing. Untreated hearing loss is on that list — not hearing aids as a vanity concession, but hearing loss as a load on the brain, which spends its reserve decoding speech rather than holding a conversation. A hearing test and, if indicated, a device.

Vision. Untreated vision loss is on the list too. A cataract getting quietly worse for six years is not just an eye problem. It is a light problem, which makes it a sleep problem.

Teeth. The bacteria of chronic gum disease have been identified in the brain tissue of Alzheimer's patients [Dominy et al., *Sci. Adv.* 2019]. Gum disease is systemic inflammation with an address. A periodontal probing once a year — not a cleaning, a probing.

What the blank line looks like

Leave this line blank and nothing happens on Tuesday. That is the whole problem with it. What happens instead is the slow version: fragmented sleep that never gets diagnosed, a body clock with no anchor, a gut barrier under daily insult so the fog gets filed under *fifty-eight*, and three front doors standing open the whole time. None of that produces an event. It produces a slope — and a slope that nobody intercepts is exactly the shape of the arc in Chapter 3. This is a line item, not a project with a completion date. You do not "finish" a premium either.

Chapter 9 — Muscle: The Reserve You Post in Advance

This is the expensive line. Nine of the ten hours are here, and most of the dollars. Four items: two strength sessions a week, a twenty-minute walk before you eat, protein first when you break the fast, and a waist measurement you take seriously. There is no fifth thing.

Two sessions, and why they are the neuroprotective item

When a muscle contracts under real load, it secretes signaling proteins into the bloodstream. Some cross into the brain and increase the primary growth factor for neurons. In plain terms, when you load a bar and stand up with it, your legs send a chemical message to your head that says *grow* [Wrann et al., Cell Metab. 2013; Moon et al., Cell Metab. 2016]. That is animal-model mechanism plus early human work, not settled human causation — but without the lift the message does not get sent, which is why “staying active” is not a substitute. Golf, a

Saturday hike, ten thousand steps: excellent things. They are not load.

The quiet loss, which is the real exposure

Past fifty, without resistance training, the average person loses roughly one percent of their muscle mass a year, accelerating toward two percent in later decades [Wilkinson et al., Ageing Res. Rev. 2018]. Quietly. Painlessly. Invisibly under good clothes. Run that forward from fifty to seventy and it is a third or more of the lean tissue you are carrying right now.

Muscle is not decoration. It is where glucose goes after a meal, which is most of what stands between you and the metabolic road in the next chapter. It is the scaffolding that decides whether a fall at seventy-eight is an inconvenience or the beginning of the end. And it is cognitive reserve: two people can carry similar amounts of underlying pathology and function very differently, and the differences travel with things like this.

The walk, and the thirty grams that follow it

Twenty minutes, five mornings, before anything crosses your lips, outdoors for the light. Overnight, insulin is at its low point, and movement in that state pulls preferentially at stored

fat. It costs nothing and doubles as your circadian anchor. Then you break the fast — with **thirty to forty grams of lean protein before anything else goes in.** Not because protein is virtuous, but because there is a threshold: below a certain dose, the signal that tells muscle tissue to maintain itself fires weakly and the body shrugs. That threshold rises with age — twenty grams was enough at thirty, thirty to forty is the floor at fifty-five. Five ounces of chicken. Five or six eggs. And then a window: most people do well eating between roughly nine in the morning and six in the evening, because insulin sensitivity is genuinely better earlier in the day and eating at nine at night puts digestion in direct competition with the sleep architecture from the last chapter.

Visceral fat, and where medication fits

The fat around your organs is not the fat under your skin. Abdominal fat is endocrine tissue: it secretes inflammatory signaling molecules around the clock, drives insulin resistance in both the body and the brain, and in a man it runs an enzyme that converts his own testosterone into estrogen — which is why the loop tightens on itself. The scale can hold perfectly steady while all of that gets worse. Measure the waist.

For some people, a licensed clinician will determine that weight-management medication is appropriate. That class of drug amplifies a satiety hormone your own gut already makes

— not a stimulant, not a cheat code — and it belongs inside the rest of this schedule, not in place of it: dropping intake sharply without the protein rule and the strength sessions costs lean tissue you cannot spare. Budget $150 to $300 a month if it is indicated, nothing if it is not.

What the blank line looks like

Leave this line blank and the ledger writes itself in the middle column of Chapter 3. Muscle comes off at one to two percent a year for twenty years. Glucose handling degrades because the place it was supposed to go is smaller. Visceral fat accumulates and produces its own inflammatory load. Then a fall at seventy-eight, or the pneumonia after the hip — the hundred days of skilled nursing that Chapter 2 told you not to count on gets consumed, and the household comes out needing hours instead of appointments, at $36,400 a year for twenty hours a week and another $1,820 for every hour on top of it, forever [CareScout 2025]. That phase change arrives through the body long before it arrives through the diagnosis.

Chapter 10 — Mitigate: Stop Paying Claims Against Yourself

This line costs essentially no hours, and it is the one nobody funds. That is not a paradox. Mitigate is not a list of things to add. It is a list of things to stop — and subtraction has no calendar entry, no equipment, and nothing to post about.

The framing I use everywhere else: **eliminate the insulting behavior.** Stop hurting yourself first. Then add what works. You have a vocabulary for this already. You do not reduce your loss ratio by buying more coverage. You reduce it by not filing claims against yourself.

The metabolic road

Nearly every road to a dementia diagnosis runs through somewhere else first, and the somewhere else is a set of numbers you can already read. The Lancet Commission's fourteen mod-

ifiable risk factors are, honestly read, mostly a cardiometabolic list [Lancet Commission 2024]. And Chapter 7 gave you the hazard ratios — 1.70, 1.72, and 1.90 for type 2 diabetes diagnosed progressively earlier, and 3.05 for obesity together with diabetes before fifty [Qi et al., PLOS ONE 2024]. Roughly a tripling, from two conditions both largely addressed by the nine hours in the last chapter.

So the four gauges here are not exotic, and your physician already draws three of them. **A1c** — Bill's came back at 5.9 at fifty-five and somebody said *borderline*, and he heard *fine*, and nobody ever mentioned it again. Know the number and the direction it has moved over three draws, not one. **LDL and the rest of the lipid panel**, on the Commission's list as of the 2024 update. **Blood pressure**, the single most modifiable item on that list and the one most likely to be sitting untreated at 148 over 90 behind the words *I feel fine*. And **waist**, which nobody's chart records. Four gauges, a baseline column, and a column twelve weeks later.

The two insults, honestly

Alcohol. I am fifty-nine, and I grew up in the same culture you did, where the after-work pour was a reward and the dinner-party wine was a social contract. But you are entitled to the number. Between three and seven drinks a week the evidence is genuinely contested and I will not pretend otherwise. Above seven, and especially in the ten-to-fourteen zone where

a lot of successful households quietly live, the picture sharpens: measurable effects on the memory structure of the brain on imaging, on sleep architecture, on hormones, and a cancer risk that climbs in roughly linear fashion. The large 2018 analysis that ended the red-wine story concluded that the level associated with the lowest overall harm is zero [GBD Alcohol Collaborators, *Lancet* 2018].

Smoking and nicotine. On the Commission's list, in every version. If this applies to you it is the largest single item on your schedule and it dwarfs everything else in this book.

Hormones and the two canaries

For the man reading this

The arteries that produce an erection are roughly one to two millimeters across. The coronary arteries are three to four. The same process that ends in a heart attack — the softening of the single layer of cells lining every vessel you own — narrows the smallest vessels first, because they have the least margin. Which is why erectile difficulty frequently precedes overt cardiovascular symptoms, commonly by two to three years and sometimes longer [Montorsi et al., Eur. Urol. 2003; Thompson et al., JAMA 2005].

That is not an embarrassment. That is a stress test your own body ran and reported the results of, years early, for free. And the brain is on the same chassis — more vascular surface area

per gram than any organ you own, the same endothelial function, the same inflammatory load, the same insulin resistance. Men who tell me their erections have softened over two years almost always tell me, in the next breath, about the word they could not find in a meeting. They believe those are two problems. They are one fire in two rooms. If you are reading this section on your husband's behalf rather than your own, you may have seen both rooms before he did.

For the woman reading this

Your body files a different report, and it is quieter, which is why it gets misread for years. It arrives as a cluster over about eighteen months, and no single piece looks like a medical event. Sleep breaks first — asleep fine, awake at two or three, ninety minutes of lying there. Then the cognitive symptoms, which frighten women more than anything else on the list: word-finding lapses, a name gone for four seconds, walking into a room and losing the reason. Then a shorter fuse and a flatness where enthusiasm used to be. Then the same calories and the same walking, and the weight relocates to the middle. Then intimacy changes and nothing gets said, because there is no script for raising it in an eleven-minute appointment.

And roughly two-thirds of Alzheimer's cases in this country are in women [Alz. Assoc. 2026]. Longer life expectancy explains part of that gap and, on the honest reading, not all of it. The menopausal transition is the leading suspect, and the argument is not settled.

Air and water

The environmental piece belongs here because it is the same shape: subtraction, and it does not hurt, which is why nobody does it. The EPA's 1989 Report to Congress measured indoor air as two to five times more polluted than outdoor air, and separate research puts the average American's time indoors at roughly eighty-seven percent [EPA 1989; Klepeis et al., JEAEE 2001]; fine particulate matter is one of the Commission's fourteen [Lancet Commission 2024]. A HEPA-plus-carbon filter in the bedroom and a countertop reverse-osmosis unit for the water is the floor. A few hundred dollars each, once. Cheap bets with plausible mechanisms on a risk no policy can lower are exactly the bets your whole career says you should be taking.

What the blank line looks like

Leave this line blank and you keep filing claims against yourself, quietly, every week. The A1c drifts from 5.9 to 6.4 over three draws that each got called *borderline*. The blood pressure nobody rechecked stays where it was. The nightly pour stays at ten. The canary — his or hers — gets explained away as stress, age, or a bad year, and the hormone picture underneath it never gets drawn. The bedroom air and the tap water stay exactly what they were. None of it costs you anything this year, which is precisely why it never gets fixed, and every one of those items is on the Commission's list of things that move the

arc in Chapter 3. This is the cheapest line on the schedule. Subtraction has no invoice. It only has a bill that arrives later.

Chapter 11 — Motivate: The Face on the Other Side of the Table

Everything so far has been an argument. If arguments worked, this chapter would not exist.

They do not. At six-fifteen on a Tuesday in March, a thing you are doing to avoid a statistic in 2044 loses to a warm bed. Every time. Not a character defect — the correct behavior of a nervous system that discounts the future, and it will beat your ledger in a straight fight for the rest of your life. So the fourth line item is the engineering that keeps the other three funded.

Three load-bearing components. A named person. A sentence about who you are. And fifteen minutes on a Sunday.

The named person

Here is the question, and it is not the one you expect. Not *why are you doing this*. **Who are you doing this for?**

Name one. Your husband. Your wife. Your daughter. Your son. Your mother, who you are already watching. Or yourself, at seventy-eight, in specific detail — still the one people call when something has to be decided. One person. One face. Someone you can picture in a room.

There is a second reason for the question, which you have already read. Chapter 4 told you who actually pays for the uninsured decade. Not the one who gets the diagnosis — the one who does not: a 63% higher four-year mortality among strained spousal caregivers [Schulz & Beach, *JAMA* 1999], chronic conditions at nearly twice the rate of non-caregivers, a decade of coming last on their own list inside a set of decisions where every call was correct [Family Caregiver Alliance]. So when I ask who you are doing this for, I am not running an exercise on you. I am asking you to notice that you already know the answer, and that it is sitting one chair over.

There is also evidence that having an answer matters on its own. In a long-running cohort of older adults, people scoring high on a validated measure of purpose in life were roughly two and a half times less likely to develop Alzheimer's over the follow-up window than those scoring low, controlling for age, education, depression, and baseline cognition [Boyle et al., *Arch. Gen. Psychiatry* 2010]. I file that under physiology, not inspiration. It behaves epidemiologically like a modifiable risk factor — and it is the only line item that costs nothing.

The sentence, and fifteen minutes on a Sunday

Once the face is named, write one sentence about who you are. Present tense. Not a goal. The distinction is not semantic. *I am trying to lift twice a week* is a behavior, and behaviors negotiate with you on bad days. *I am someone who lifts* is an identity, and missing it is then dissonant, and your brain hates dissonance enough to route you back.

That is where the line on the cover of my other book actually lands, and it is not a slogan. **Don't lose your identity and your dignity while you still have a choice.** The ledger is about the money. This is about the choice, and the choice is exercised at the level of the sentence you use to describe yourself.

That is the schedule of benefits. Four lines, ten hours, and a premium denominated in something no carrier can reprice and no underwriter can decline. All that is left is signing it.

PART IV — SIGNING

Chapter 12 — Begin with the End in Mind

There is a paperback somewhere in your house with a tan cover and a cracked spine. You read it in 1989, on a plane, probably. Habit 2. *Begin with the end in mind.* Covey's exercise was to sit at the back of the room at your own funeral, listen to four people speak, and ask what you would want them to be able to say. Then work backward from that.

And there is exactly one thing you never ran the exercise on. The instrument that was supposed to be doing the living.

You planned the estate. You did not plan the mind that was going to spend it. Every document in that fireproof box quietly assumes a version of you at seventy-eight who can still read it, still sign it, still be the one in the room whose opinion settles the question. Your advisors did their half well: the documents exist, they are correct, and they will do what they were drafted to do. The half nobody could put on that checklist is the health of the person the documents are written around. That half is yours.

That assumption is what this entire book has been auditing.

The end, actually stated

So state it. Not the funeral — that is the wrong end for this exercise. The end that decides everything is the decade before it. Here is a draft you can steal and edit:

The last ten years, sharp. On my feet. In my own house. Making my own decisions, and still the one people call when something has to be decided. And the one I married spends that decade as my partner, not as my nurse.

Then notice the thing that took me an embarrassingly long time to see myself. The destination is a mind. Not the money the mind manages. Not the body that carries it around. Not the house, which is only in the sentence because being in your own house at eighty-four requires that you can still be trusted to run a stove. Every item in that paragraph is downstream of one organ, and it is the organ nobody built a policy for.

That is why the schedule in Part III is shaped the way it is. Muscle, Mitigate, and Motivate are the three line items. Mind is not a fourth item sitting alongside them — it is what the other three purchase. *Begin with the end in Mind.* Both readings at once: start from the destination, and understand that the destination is the mind itself.

The face and the sentence

Chapter 11 gave you the machinery. This is where you run it. One person — not a category, not "my family," which is four people and therefore nobody. One face you can put in a room. Write the name on the line. Physically. In the book, in pen.

I am doing this for: ____.

Bill and Cindy, at fifty-five

If the word has already been said in an office in a building with a parking garage, then the arithmetic in Part I is not a forecast for you — it is an invoice. Ask a licensed clinician, in an actual evaluation, what medical support is available to a household already past the diagnosis, and read Chapter 4 again, because the second patient in your house is the only person in the marriage not being cared for.

But most of you are Bill and Cindy at fifty-five, and either chair at that table can turn out to be Bill's. Thirty pounds up. An A1c somebody called borderline, which got heard as fine. Sleeping badly and calling it a personality trait. A knee nobody had looked at, and a Thursday. There is nothing wrong with either of you that anyone has put a name to. That is precisely the problem: nothing has a name yet, which is the only window in which any of this works.

In the story, Cindy tried twice. Once in the car coming home from a wedding, and once in the kitchen on a Sunday, and both times she let it go, because Bill had a business and a bad shoulder and a hundred reasons to hear it as criticism. She was not being timid. She was being married, which is a job that involves a great deal of judgment about which Tuesday is the right Tuesday.

She never got a third try in that story. In yours, somebody just did — because this book was handed to you, or left on the counter, or mailed without a note. Or because you are the one who keeps the folder in your house, and you bought it yourself.

Either way, you are the one holding it, so finish the conversation. If it was handed to you, do not promise anything; you have made promises about your health before and you both remember how they went. Say the smaller, truer thing instead:

I read it. I'm going to take the seven-minute assessment tonight, and I'll show you what it says.

If you are the one who did the handing — or the one who has been carrying the folder alone — take it yourself first. The risk in this book belongs to both chairs at that table, and for a woman the lifetime number is the larger one. Then put your result on the table, and ask for the other.

That is the whole conversation. It is not dramatic. It never is.

Chapter 13 — The First Thirty Days

Coverage does not begin when you decide to buy it. It begins when the first premium clears.

Week 1 — Baseline. Take the assessment at **my4mlife.com/assessment**: twenty questions, about seven minutes, tonight. It gives you a score out of 100, the band you are in, and your top three priorities, ranked. It is free, and it is also the door — taking it sets up your account, so the app, the Logbook, a copy of *Begin with the End in Mind*, and the weekly Zooms come with it. Then set the two anchors, both about the clock: ten minutes of outdoor light within an hour of waking, not through a window, and one fixed wake time seven days a week. Not a bedtime — a wake time; the bedtime follows on its own. Put the two strength sessions in the calendar with names, like meetings: *Monday 6:15 — Strength*. You do not miss meetings. You miss intentions. Start the walk on day three. On day seven, tell the named person, out loud, to their face.

Week 2 — The kitchen. One rule at first: **protein first.** You break the fast after the walk, with thirty to forty grams of

lean protein before anything else goes in. That one rule reorganizes the rest of the day's eating without your having to think about it. And the grocery list changes before the meals do: you start by shopping differently, once, and then the kitchen makes the decision for you all week. Then remove one insult from Chapter 10 — the second drink, the eleven-o'clock screen. One. Not four. On Sunday, run the fifteen minutes of planning.

Week 3 — Load, and the labs. The sessions get heavy: compound movements, loaded so the last two repetitions are genuinely difficult. If you have not trained in years, spend the week learning the movements at a weight that feels insulting. Get the panel drawn if it is indicated — a lab draw and an actual evaluation by an independent licensed clinician, read against what is optimal for you rather than a reference range built from a population that is not aging well. Whether anything warrants prescription support is a licensed prescriber's determination after seeing you, not this book's; if you do not have a clinician, start at **my4mlife.com/consult**. And book the three appointments: hearing test, eye exam, periodontal probing.

Week 4 — The household ledger, and the advisor meeting. This is the week most people let slide. Do not. Run your own number at **my4mlife.com/go/uninsured-decade** — your age, your spouse's, your state, roughly where you sit financially — and print the sheet. Then sit down with your spouse. Then book an hour with the advisor, attorney, or CPA who handed you this book. Bring the printout and the

coverage inventory from Chapter 1, and review four things with them: what the plan assumes about a long custodial-care event, what LTC or hybrid coverage is available and appropriate, the survivor's position if it lands, and whether the powers of attorney and healthcare directives are current and signed. That is their half, it is the half with legal deadlines on it, and it is not a conversation to have after a diagnosis. Yours is the schedule in Part III. Then show up to one Zoom.

Day 30. Here is what will not have happened: you will not have lost thirty pounds, your A1c has not moved much, nobody at work will comment. Here is what will have happened, in most people. **You will be sleeping.** Not perfectly, but consolidated — you go down, and the next thing is the alarm, not the ceiling at one-forty and again at four. It is almost always the first thing to move, and almost always what convinces the skeptic in the house that the rest of this is real, because it is not a number somebody handed over. It is your own Tuesday, arriving different. That is the dividend, arriving early, on a premium you have been paying for one month. The other payout — the ledger that never gets written — you will never be able to prove you collected.

The 30-Day Ledger

	Item	Date done
☐	Assessment taken — score, band, top three	_____
☐	Wake time fixed, seven days	_____
☐	Ten minutes of morning light	_____
☐	Two strength sessions on the calendar, by name	_____
☐	First fasted morning walk	_____
☐	Named person told, out loud	_____
☐	Identity sentence written down	_____
☐	Protein first — thirty to forty grams after the walk	_____
☐	Grocery list rewritten once	_____
☐	One insult removed	_____
☐	Fifteen minutes on a Sunday	_____
☐	Two sessions run, loaded	_____
☐	Labs drawn / evaluation booked	_____
☐	Hearing, eyes, periodontal probing booked	_____
☐	Own ten-year number run and printed	_____
☐	Household ledger conversation held	_____
☐	Hour booked with the advisor who gave you this book — printout, coverage inventory, POA/directives	_____

The Premium

The premium: *roughly ten hours a month and roughly eight to ten thousand dollars a year, most of which is money already in your budget under different headings.*

What it buys: *two payouts. A contingent one — lower odds on the decade in Chapter 3, at the conservative assumption I defended in Chapter 7, stated as odds and never as a promise. And a non-contingent one that starts inside six weeks: sleep, strength, the four-o'clock, presence, an active retirement decade, and a spouse whose last decade is spent as your spouse.*

What it does not buy: *immunity. Nothing in this book prevents, treats, cures, or reverses dementia, and no one who tells you otherwise is doing math. It lowers the odds. That is the only sentence I have sold you, it is the same one I started with, and it is the last one.*

Signing

Every policy you have ever bought was signed at a table by a version of you who did not know whether it would ever be needed. That is what a signature is for: a decision made in advance, by the person who is thinking clearly, on behalf of the person who will not be. You did it at thirty-four with the term life, at forty-one with the disability, at fifty-two with the buy-sell, and every time you were signing on behalf of somebody who was not in the room.

This one is the same act. The only difference is that the person you are signing on behalf of is you, at seventy-eight, and there is no carrier on the other side of the table to countersign it. There is no policy number. There is no folder to put it in. There is a name, a sentence, and a Tuesday morning that is either going to happen this week or not.

I told you on page five that I would not claim at the end that this pays for itself, and I have not. I said I would show the arithmetic with nothing hidden, and I did, including the part where it comes up short. What I will tell you is what I believe after thirty years of watching households arrive at this decade either provisioned or not: this is the one exposure on your schedule where the premium is denominated in something no

underwriter can decline you for, no carrier can reprice, and no market can leave.

You are the only one still writing this policy.

So sign it. Tonight, in seven minutes, at my4mlife.com/assessment — and then get up tomorrow and go for a walk before you eat.

Don't lose your identity and your dignity while you still have a choice.

— Dr. TJ

The Assessment

Everything in this book routes to one page.

my4mlife.com/assessment

Twenty questions. About seven minutes. Free, and there is nothing to buy at the end of it.

What it gives you back, on screen, immediately:

- A **MindSpan Score out of 100** — your baseline, which is the number you do not currently have.
- The **band** you are in, in plain language.
- Your **top three priorities**, ranked. Not a list of twenty things. The three that will do the most for you, in order.

What happens next. Your full results arrive by email, along with what comes with being a Protégé, at no charge: the **app**, which runs the four-month program week by week; **The Logbook**; a copy of ***Begin with the End in Mind***, the full-length book behind the framework in Part III; and **weekly Zooms** with other people running the same schedule.

If a licensed clinician needs to be involved — labs, hormone support, anything requiring a prescriber — that is a separate

conversation at **my4mlife.com/consult**, with an independent licensed medical practice, after an actual evaluation. If you want your own household's ten-year number instead of the national median in Chapter 3, run it at **my4mlife.com/go/uninsured-decade** and print the sheet for your advisor. To be clear: the assessment is the health-span half; the money half stays with the advisor who handed you this book.

Take the assessment tonight. The rest can wait until the weekend. That one cannot, because it is the only line on the schedule that has to happen first.

About the Author

Dr. TJ has spent more than thirty years in clinical practice and now works as a health-span educator, writing and speaking about cognitive longevity for the people who make their household's decisions. He lives and works in Fort Worth, Texas.

He is the founder of **My4MLife**, and the architect of the 4M framework — Mind, Muscle, Mitigate, Motivate — which an assembled team of clinicians, coaches, and researchers built into a working program under his guidance. He does not practice medicine; all medical care, evaluation, and prescribing referenced in his work is provided by independent licensed medical practices.

He is the author of ***Begin with the End in Mind*** and ***Count Yourself Skinny***.

my4mlife.com · my4mlife.com/assessment

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